

Results

BIOPSY OR §

Patient Information

Patient Name

Sex

DOB

Result Information

Status

Provider Status

Final result

Reviewed

Entry Date

Component Results

Component

Lab

Surgical Pathology

Patient Name

MR#

Specimen

ICD-O-3

Carcinoma, urothelial, NOS

8120/3

Site: bladder, wall, NOS

C67.9

8-27-12

PO

Final Diagnosis

A. Right ureter, biopsy:

-Segment of benign ureter, negative for tumor

-Frozen section diagnosis confirmed

B. Left ureter, biopsy:

-Segment of benign ureter, negative for tumor

-Frozen section diagnosis confirmed

C. Left external and internal lymph nodes, resection:

-Benign fibrovascular tissue, negative for tumor

-No lymph nodes identified

D. Left internal and external iliac lymph node, resection:

-Single lymph node with metastatic urothelial carcinoma, 1.8 cm largest diameter

(1/1)

E. Left internal and external iliac lymph nodes, resection:

-Eight benign lymph nodes and fibroadipose tissue, negative for tumor (0/8)

F. Left obturator lymph node, resection:

-Three benign lymph nodes and fibroadipose tissue, negative for tumor (0/3)

G. Left common iliac lymph, resection:

-One benign lymph nodes and fibroadipose tissue, negative for tumor (0/1)

H. Right internal and external iliac node, resection:

-One benign lymph nodes and fibroadipose tissue, negative for tumor (0/1)

I. Right obturator lymph node, resection:

-Benign fibroadipose tissue, negative for tumor

-No lymph nodes identified

J. Urinary bladder and prostate, radical cystoprostatectomy:

-Invasive high grade urothelial carcinoma with glandular, micropapillary and sarcomatoid differentiation, 4.0 cm. involving bladder diverticulum

-Tumor invades into perivesicle tissue (pT3a)

see TCGA Pathologic Diagnosis Discrepancy Form

Criteria	Yes	No
Diagnosis Discrepancy	☒	☐
Primary Tumor Site Discrepancy	☐	☐
HPV Discrepancy	☐	☐
Other Malignancy History	☐	☐
Qual/Synchronous Primary History	☐	☐
Case is Final	☒	☐
Reviewer Initials	Date Reviewed: 8/26/12	

-Surgical resection margins are negative for tumor
-Incidental prostatic adenocarcinoma, Gleason grade 3+3=6, organ confined (pT2a)
and negative surgical margins
-See CAP cancer template for further details

Specimen
Bladder and prostate

Procedure
Radical cystoprostatectomy

Tumor Site
Left lateral wall with involvement of diverticulum and extension to posterior
wall, right lateral, anterior walls and prostatic urethra

Tumor Size
Greatest dimension: 4.0 cm
Additional dimensions: 4.0 x 3.0 cm

Histologic Type
Urothelial (transitional cell) carcinoma with glandular, micropapillary and
sarcomatoid differentiation

Associated Epithelial Lesions
None identified

Histologic Grade
High grade

Tumor Configuration
Ulcerated

Microscopic Tumor Extension
Perivesical fat

Margins
Margins uninvolved by invasive carcinoma

Lymph-Vascular Invasion
Present, extensive

Pathologic Staging (pTNM)
Primary Tumor (pT)
pT3a: Tumor invades perivesicle fat, microscopic

Regional Lymph Nodes (pN)
pN1: Single regional lymph node metastasis in the true pelvis (hypogastric,
obturator, external iliac or presacral lymph node)
Specify: Number examined: 14 (see specimens C through I)
Number involved (any size): 1

Distant Metastasis (pM)
Not applicable

Additional Pathologic Findings
Evidence of previous biopsy
Incidental adenocarcinoma of prostate (see CAP template below)

Ancillary Studies:
Block J18 has been sent for ancillary studies, see separate report

CAP cancer template for prostate cancer:

Procedure

Radical cystoprostatectomy

Prostate Size

Weight: Not applicable (part of bigger specimen)

Size: 3.7 x 5.3 x 1.3 cm

Lymph Node Sampling

Pelvic lymph node dissection (see specimens C through I)

Histologic Type

Adenocarcinoma (acinar, not otherwise specified)

Histologic Grade

Gleason Pattern

Primary Pattern: Grade 3

Secondary Pattern: Grade 3

Tertiary Pattern: Not applicable

Total Gleason Score: 6

Tumor Quantitation

Proportion (percentage) of prostate involved by tumor: 1%

Extraprostatic Extension

Not identified

Seminal Vesicle Invasion

Not identified

Margins

Margins uninvolved by invasive carcinoma

Treatment Effect on Carcinoma:

Not identified

Lymph-Vascular Invasion

Not identified

Perineural Invasion

Not identified

Pathologic Staging (pTNM)

Primary Tumor (pT)

pT2a: Unilateral, involving one-half of 1 side or less (left side)

Regional Lymph Nodes (pN)

pN0: No regional lymph node metastasis

Specify: Number examined: 14 (see specimens C through I)

Number involved: 0

Distant Metastasis (pM)

Not applicable

Additional Pathologic Findings:

High grade prostate intraepithelial neoplasm

Ancillary Studies:

Block J28 can be sent for ancillary studies if needed.

K. Appendix, appendectomy:

Benign appendix without significant histopathologic abnormality

L. Left obturator, resection:

-Benign fibroadipose tissue, negative for tumor

-No lymph nodes identified

The above pathology diagnosis incorporates data elements as outlined in the CAP Cancer Protocols and Checklists which are based on the AJCC/UICC TNM, 7th edition.

Clinical History

Bladder cancer.

Radical cystectomy, lymph node dissection, ileal neobladder, possible ileal conduit, all other indicated procedures.

Source:

A: Right ureter

B: Left ureter

C: Left external and internal Lymph node

D: Left internal and external iliac Lymph node

E: Left internal and external iliac lymph nodes

F: Left obturator lymph nodes

G: Left common iliac lymph nodes

H: Right internal and external iliac lymph nodes

I: Right obturator lymph nodes

J: Bladder and prostate

K: Appendix

L: Left obturator

Gross Description

Specimen A, received fresh labeled with the patient's name and designated 'right ureter, silk suture distal end' is a tubular segment of tissue 1.2 cm in length and 0.5 cm in diameter. The proximal margin is submitted for frozen section diagnosis as A1.

Specimen B, received fresh labeled with the patient's name and designated 'left ureter, chromic stitch distal end' is a tubular segment of tissue 1.3 cm in length and 0.5 cm in diameter. The proximal margin is submitted for frozen section diagnosis as B1.

Specimen C, received fresh labeled with the patient's name and designated 'left external and internal lymph nodes' is a 4.5 x 1.0 x 0.4 cm portion of adipose tissue. There is a 1.0 x 0.8 x 0.6 cm with a possible 1.0 x 0.8 x 0.6 cm lymph node. The node is submitted entirely for frozen section as C1.

Specimen D, received fresh labeled with the patient's name and designated 'left internal and external iliac lymph node' is a 1.8 x 1.5 x 1.2 cm tan node. The cut surface is tan slightly bulging firm parenchyma. A section is submitted for frozen section diagnosis.

Section key: The specimen is submitted entirely as follows:

D1 - frozen section

D2 - remainder of nodule

Specimen E, received in formalin labeled with the patient's name and designated 'left internal and external iliac lymph node' is 3.3 x 3.5 x 1.0 cm of fragmented fibrofatty tissue. There are seven possible nodes, 0.5 cm-1.0 cm. The nodes are submitted entirely as follows:

E1 - four whole nodes

E2 - two nodes, one inked blue and each bisected

E3 - one node, bisected

Specimen F, received in formalin labeled with the patient's name and designated 'left obturator lymph node' is 3.3 x 3.0 x 1.0 cm of fragmented fibrofatty tissue. There are three possible nodes, 0.5cm-1.5 x 0.8 x 0.5 cm. The nodes are submitted entirely as follows:

F1 - two nodes, one inked blue and each bisected

F2 - one node, bisected

Specimen G, received in formalin labeled with the patient's name and designated 'left common iliac lymph node' is a 2.0 x 1.0 x 1.0 cm portion of adipose tissue. There is a 1.2 x 0.8 x 0.8 cm tan node. The node is bisected and submitted entirely as G1.

Specimen H, received in formalin labeled with the patient's name and designated 'right internal and external iliac lymph node' is 4.0 x 3.0 x 1.0 cm of fibrofatty tissue. Sectioning into this specimen shows approximately one-half of the specimen as muscle fibers. There is one possible node, 2.0 x 0.6 x 0.4 cm. The node and representative muscle with fibrous tissue submitted as follows:

H1 - one node, bisected

H2 - muscle with white fibrous tissue

Specimen I, received in formalin labeled with the patient's name and designated 'right obturator lymph node' is 1.8 x 1.5 x 0.8 cm of fibrofatty tissue and muscle. A discrete lymph node is not identified. The specimen is submitted entirely as I1.

Specimen J:

Type of specimen: Urinary bladder, ureter stumps, prostate, and seminal vesicles
The patient's name and case number on the specimen container match the accompanying paperwork and cassettes.

Procedure: Radical cystoprostatectomy

Fixative: Formalin

Dimensions: Bladder 4.3 cm dome to neck x 4.2 cm left to right x 3.5 cm anterior to posterior; ureter stumps, right 3.5 cm in length and 0.4 cm in diameter; left ureter 3.8 cm in length and 0.4 cm in diameter; prostate 3.7 cm apex to base, 5.3 cm left to right, 1.3 cm anterior to posterior

Description of findings:

Tumor: There is an ulcerated, focally fluffy tan to brown mass, 4.0 cm dome to neck x 4.0 cm left to right x 3.0 cm anterior to posterior. The mass is on the posterior wall extending into the anterior, right and left wall. There is a diverticulum in the left bladder extending to a depth of 2.0 cm, with mass invading into the wall and to within 0.2 cm of the perivesicular adipose tissue (J17-J18).

Involvement of important adjacencies: The tumor possible invades into the perivesicular adipose tissue, with no other structures involved.

Uninvolved mucosa: Tan and focally edematous in the right dome.

Prostate, seminal vesicles and vasa deferentia: The prostate is sectioned apex to base into six slabs. The parenchyma is tan, with pale foci and no discrete mass.

Ink code:

Right prostate - orange

Left prostate - blue

Anterior bladder - green

Posterior bladder - black

Section key:

J1 - urethral margin

J2 - bladder neck right

J3 - bladder neck left

J4 - right ureteral orifice, blue ink on ureter

J5 - left ureteral orifice, blue ink on ureter

J6-J8 - tumor posterior wall, each full thickness section

J9-J10 - tumor right wall, each full thickness section

J12-J13 - tumor left diverticulum, each full thickness sections

J14-J15 - tumor left wall, full thickness sections

J16 - tumor left diverticulum

J17-J18 - tumor left diverticulum to within 0.2 cm of perivesicular adipose tissue

J19-J20 - tumor anterior wall

J21 - prostate right seminal vesicle - prostatic interface, vas deferens

J22 - slab 1, apex prostate

J23 - slab 3, right posterior prostate

J24 - slab 6, right posterior prostate

J25 - prostate left seminal vesicle - prostatic interface, vas deferens

J26 - slab 1, left apex prostate

J27 - slab 2, left posterior prostate

J28 - slab 6, left posterior prostate

Specimen K, received in formalin labeled with the patient's name and designated 'appendix' is a vermiform appendix with mesoappendix. The appendix is 7.7 cm in length and 0.5 cm in diameter. The mesoappendix is 9.0 x 2.4 x 0.8 cm. The appendiceal serosa is tan, smooth and glistening. The lumen ranges from 0.1 cm to 0.2 cm and is filled with brown fecal material. At the distal tip there is a pocket of fecal material, with unremarkable mucosa. Representative sections are submitted, with the proximal margin inked blue as K1.

Specimen L, received in formalin labeled with the patient's name and designated 'left obturator' is a 2.3 x 1.5 x 0.6 cm portion of fibrofatty tissue with muscle, with multiple staples and sutures. The specimen is vascular without a discrete mass or lymph node. The specimen is submitted entirely as L1.

Microscopic Description

The H and E stain quality is acceptable.

A-J. The microscopic findings are reflected in the diagnoses rendered.

Intraoperative Diagnosis

Time in: Time out:

A1-FS, Right ureter: Negative for carcinoma or high grade dysplasia.

Time in: Time out:

B1-FS, Left ureter: Negative for carcinoma or high grade dysplasia.

Time in: Time out:

C1-FS, Left external and internal lymph node, peripheral nerves and fibrovascular tissue with inflammation, negative for carcinoma.

Time in: Time out:

D1-FS, Left internal and external iliac lymph node: Positive for metastatic carcinoma.

All Reported to

Electronically Signed Out

Result History

BIOPSY OR SURGICAL SPECIMEN ([REDACTED])

Collection Information

Specimen Source
Other

Date and Time

Accession #

Lab Information

Reviewed by List

Status:

This order is currently not shared in ^

Order

Biopsy or Surgical Specimen

Patient Information

Patient Name

Sex

DOB

Visit Information

Date

Time

Department

Provider

Visit #

FIN #

Order Information

Order Date and Time

Ordering User

Department

Order

Order Name

Code

Order Number

Biopsy or Surgical Specimen

Order Details

Frequency

Duration

Priority

Order Class

ONE TIME ORDER TASK 1 Occurrence

Routine

Nurse Collect

Order Providers

Authorizing Provider

Encounter Provider

Original Order

Ordered On



Ordered By

Collection Information

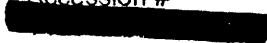
Specimen Source

Other

Date and Time



Accession #



Reprint Requisition

Reprint Requisition (

